

Before You See Your Doctor

A worksheet to fill in by hand and bring to your appointment — July 2026

Your doctor fills out the medical report, called the DS2444B. It is long, and it asks for detail that is hard to reconstruct from clinic notes in a short appointment. This worksheet is for you to fill in beforehand, in your own handwriting, and hand to your doctor at the start of the visit.

It is not a form the government issues and it is not sent anywhere. It exists so that the person who knows your daily life best — you — can give your doctor the raw material, and so your doctor can spend the appointment on clinical judgement instead of on reconstructing your history.

Write plainly, and do not tidy it up

The most useful answers are ordinary and specific. “I nearly dropped my coffee cup because my arm gave out” is worth more than “I have weakness in my arm.” Describe your worst days as well as your ordinary ones. Nothing here is complaining, and nothing here is too small.

Do not leave anything out because you think it is unrelated

Conditions that seem minor on their own can matter a great deal when added together. The assessment looks at your combined situation, not one diagnosis at a time.

1. Your details

Your name	Date of birth
Doctor's name	Seeing this doctor since (year)

2. Your conditions

List everything, including conditions you have had for years and ones that come and go. Use extra paper if you run out of rows.

Condition or diagnosis	Started (year)	What it stops you doing
	<i>roughly is fine</i>	<i>the everyday effect, in your words</i>

3. What a bad day looks like

Walk through one from waking up to going to bed. What goes wrong, what you cannot finish, what you need help with, how many times you have to stop or lie down.

4. What an ordinary day looks like

Not your best day. An average one. This matters because the difference between the two is part of what the report has to capture.

Say how often, not how bad on average

If your condition comes and goes, write the pattern rather than a middling answer. “Three bad days most weeks, sometimes a whole week” tells the story. “It varies” does not. Unpredictability is itself a barrier to working, and it only counts if it is written down.

5. How often are the bad days?

How many in a typical week or month. How long they last. Whether you can tell one is coming.

6. Things you used to do and cannot do now

Work, housework, cooking, driving, shopping, hobbies, seeing people, caring for others. Say roughly when each one stopped.

What you used to do	When it stopped	What stops you now

7. Help you need from another person

Anything someone else does for you, or that you cannot do alone or safely alone. Include help you would need but do not have.

8. Treatments you have tried

Include anything stopped, refused, or never started, and the reason. If a specialist advised against something, or a waitlist is long, write that here.

Treatment, therapy or medication	Result, or why it stopped
	“did not help” and “made things worse” both matter

If you turned down a treatment, write down why

Declining a treatment can be read badly later by someone who was not in the room, as though help was refused. If a specialist raised concerns about the risks, or the cost or the travel put it out of reach, or your condition itself makes it hard to keep up with, say so here. Ask your doctor to record the reason in the report. Same facts, very different reading.

9. Work history, and what ended it

What you did, for how long, and what happened. If you tried to keep working and could not, describe what that cost you.

10. Anything else

Anything that has not fit above. Falls, hospital visits, sleep, pain at night, memory, mood, or how you manage on your own.

A note for your doctor

Hand this page over with the completed worksheet

Your patient is applying, or reapplying, for Assured Income for the Severely Handicapped (AISH), and the form you are being asked to complete is the DS2444B medical report. The preceding pages were filled in at home, to save you reconstructing daily function during the appointment.

The determination turns on whether there is a severe and permanent impairment that substantially limits the ability to earn a livelihood. Three things carry disproportionate weight and are commonly under-recorded:

Variability, recorded as a range

Where function fluctuates, the range is the finding. A rating written as “severe on flare days, mild between” carries the file. A blended middle rating erases the evidence. Frequency, duration and predictability of episodes are load-bearing.

Permanence, and whether options are exhausted

Files are commonly refused on the theory that further treatment might restore capacity. Where that is clinically not the case, stating it plainly closes the question. Where a treatment is nominally available but inaccessible within a clinically relevant timeframe, that is worth recording as such.

Accommodation

Language describing what accommodations would enable can read as capable with support. Where it is clinically true that no accommodation renders sustained competitive employment reliable, saying so directly is the clearer finding. An employer can adapt a workstation; none can adapt to unpredictable incapacity.

Where adherence has been limited, the reason belongs on the record. An unexplained note of non-compliance can undermine an otherwise strong file, whereas cost, side effects, access, or the disability itself impairing self-management are findings in their own right.

Please complete the report to the true clinical picture and no further. Accuracy is the position that withstands review. This worksheet supplies context, not conclusions.

A fuller question-by-question guide to the DS2444B is available free at albertadisabilitysystembreakdown.netlify.app. The official form is at open.alberta.ca.

The Alberta Disability System Breakdown — albertadisabilitybreakdown@outlook.com — general information, not legal or medical advice. Free to share, print and post.